

Ethical Issues in Pediatrics

Children are immature and depend on their parents or guardians emotionally and financially. They cannot make informed decisions about their care. Children must be protected from the consequences of unwise decisions that they or others make. It is tragic if a child dies or suffers serious harm because a simple, effective medical treatment was not provided.

HOW ARE ETHICAL ISSUES IN PEDIATRICS DIFFERENT?

CHILDREN ARE NOT AUTONOMOUS

Because young children cannot weigh risks and benefits, compare alternatives, or appreciate the long-term consequences of choices, they are incapable of making informed decisions. Hence, autonomy is less important in pediatrics than in adult medicine. Children's objections to beneficial medical interventions do not have the same ethical force as adults' informed refusals. Because children are immature and vulnerable, they need an adult to make decisions for them and to look after their best interests. Parents are presumed to be the appropriate decision makers for their children (1).

PHYSICIANS SHOULD BE ADVOCATES FOR CHILDREN

Doctors have a unique opportunity to identify when a child's health and well-being are jeopardized by their parents' decisions or actions. Physicians therefore have special responsibilities in these situations to prevent suffering serious, long-lasting harm.

RESPECT CHILDREN'S POTENTIAL TO BECOME AUTONOMOUS

Children's potential to become autonomous adults deserves respect. Parents mold children, and parental values deserve great deference. However, when children reach maturity they might choose different values from their parents'. Physicians need to help ensure that parental decisions do not close off a child's open future as a unique person. As children grow, they become capable of making informed decisions, and their involvement in care should increase. Physicians need to provide children with information about their conditions and opportunities to participate in decisions about their care, to the extent it is appropriate developmentally (1).

WHAT STANDARDS TO USE IN DECIDING FOR CHILDREN?

Because children cannot make informed decisions, beneficence—acting in the child's best interest—is the primary ethical guideline in pediatrics.

CHILDREN'S BEST INTERESTS

The concept of "best interests" emphasizes that children are persons separate from their parents, with their own interests and rights. Generally parents' decisions and ongoing involvement in a child's care

promote the child's best interests. Assessments of a child's best interests, however, may be contested, with reasonable people disagreeing over which outcomes and risks are acceptable, and how to weigh the benefits and burdens of interventions. Moreover, promoting some of the child's interests might set back other interests of the family.

A child's best interests include both the duration and quality of life. Although quality-of-life judgments seem unavoidable, they might also be ethically problematic. It is difficult to predict a child's future quality of life. Healthy people tend to underestimate the quality of life of persons with chronic illness. Although some people believe that Down syndrome is a fate worse than death, many children with this condition experience happiness and are prized by their parents. Chapter 4 discusses best interests in detail.

CHILDREN'S PREFERENCES

To the extent that children have the capacity to make informed decisions about their medical care, their choices should be respected. Chapter 10 discusses how to determine whether a patient has the capacity to make medical decisions.

Even when children are not capable of giving informed consent, their *assent* to interventions is still ethically important if it is developmentally appropriate. It is disturbing to force interventions on a child who can understand what will be done and is actively resisting it. A child's objections, however, are not necessarily decisive. For instance, a child who objects to shots should still receive immunizations. Forced therapy, however, becomes problematic if children are older, the effectiveness of the intervention is uncertain, or the side effects are more probable, more serious, or longer lasting. The physician should listen to and respond to the child's reasons for dissenting from treatment. If interventions are carried out despite the child's objections, it is appropriate for the doctor to apologize to the child.

PARENTS' AND OTHER FAMILY MEMBERS' INTERESTS

Although the child's best interests are of primary concern, parents and other family members have interests that must also be taken into account. What is best for an individual child can be understood only in the context of what is best for the family as a whole or for other family members. Parents cannot be expected to devote all their energy and resources to one child, even though they should make some sacrifices; for example, parents might choose not to buy a house in the best school district, but instead to live closer to their jobs.

WHO SHOULD DECIDE FOR CHILDREN?

THE PRESUMPTION OF PARENTAL DECISION MAKING

Parents are presumed to be the appropriate decision makers for their children. Generally, love motivates them to do what is best for their children. In addition, parents have long-term relationships with and obligations to their children. In most cases, parents concur with physicians' recommendations—for example, agreeing to antibiotics for strep throat, bronchodilators for asthma, and surgery for appendicitis.

American culture prizes parental responsibility, family integrity, and strong parent–child relationships. Parents or guardians have considerable latitude, but not unlimited discretion, in raising children. Within limits set by society, parents have discretion to inculcate their values in children and to make choices for rearing their children. For example, children must attend school, but parents may choose the type of school.

Physicians speak of *parental permission* rather than consent to distinguish what people may decide for themselves from what they may decide for their children. Although informed adults have a right to refuse any medical intervention, parents do not have absolute power to refuse care for their children (1). As noted, parental permission should be supplemented with the child's assent when developmentally appropriate (1).

Parents commonly ask physicians what they would do if it were their child. Physicians need to clarify what the parents are asking (2). They might be asking what care would optimize their child's outcome, seeking the physician's medical judgment and reasoning. Alternatively, parents might ask whether they are making the right choice. The physician's response needs to be supportive and compassionate. If the parents want to know what the physician personally would do, it is helpful for physicians to describe the process of decision making they would use, including talking with relatives and friends, as well as what factors they would consider. If parents still want to know what the physician would do, then it is appropriate to offer a recommendation based on the parents' values and goals, which may differ from the physician's.

Emergencies

In an emergency, when a parent or guardian is not available and a delay in treatment would jeopardize the child's life or health, the physician should act in the child's best interests and immediately provide appropriate treatment without waiting for parental permission (3).

Exceptions to Parental Decision Making

Some parents are estranged from their children or unwilling to be involved in their care. Other parents lack the capacity to make informed decisions, for example, because of substance abuse or developmental disability. Strictly speaking, parents should make decisions for children unless a court has appointed someone else as guardian. Informal arrangements, however, are often made for another relative to make decisions when parents are absent or incapable of making decisions.

ADOLESCENT PATIENTS

As children mature, they develop the capacity to make informed decisions about their health care. Most states allow 18-year-olds to give informed consent or refusal to medical care without parental involvement. Younger minors may make their own decisions about health care because of their status or the condition for which treatment is sought (4, 5). State laws regarding the medical care of adolescents balance several countervailing policy goals: fostering access to treatment for important public health problems, respecting adolescents who are functionally adults, and encouraging parental involvement in their children's care. Because statutes vary according to state and medical condition, physicians need to know the laws in their jurisdiction.

Status of the Minor

Mature minors are capable of giving informed consent. Ethically, mature minors should be allowed to consent to or refuse medical treatment. Generally, adolescents above 14 or 15 years of age have such decision-making capacity, but younger children commonly have difficulty entertaining alternatives, appreciating the consequences of decisions, and appraising their future realistically. In most states, a court must declare an adolescent a matured minor.

Emancipated minors are recognized as *de facto* adults because of marriage, service in the armed forces, or living apart from parents and managing their own finances. Many states require a judicial hearing and declaration of emancipation by the courts (5).

Treatment of Specified Conditions

Most states allow minors to obtain treatment without parental permission for sensitive conditions, such as sexually transmitted infections (STIs), contraception, pregnancy, sexual assault, substance abuse, and psychiatric illness (6). The rationale is not that adolescents who seek treatment for such conditions are making informed decisions—indeed, these conditions might impair judgment or result from unwise choices. Instead, the justification is that requiring parental permission would deter many adolescents from seeking treatment for important, treatable public health problems. Even when adolescents consent to such care themselves, it is generally in their best interests to involve their parents in their subsequent care.

Parental Requests for Treatment

Parents might request that the physician test an adolescent for illicit use or pregnancy without telling the child (7). Although parents are naturally concerned, surreptitious testing is unacceptable because it violates the adolescent's emerging autonomy, creates mistrust and suspicion in the family, and undermines the physician–child relationship (7).

PHYSICIAN–CHILD–PARENT–RELATIONSHIP

Disclosing of information to children, protecting confidentiality, and truth-telling show respect for children, lead to beneficial consequences, and foster trust in the medical profession.

DISCLOSURE OF INFORMATION TO CHILDREN

Physicians should provide children pertinent information about their care in terms they can understand. Children who cannot understand medical details might still want to know what will be done to them. Doctors should also obtain the child's assent if this is developmentally appropriate.

Some parents do not want their children to know about serious diagnoses, such as cancer or HIV infection (8). Physicians should elicit the parents' concerns and fears. Parents might believe that the child will not be able to handle bad news or that peers will reject the child. Physicians can explain how children usually cope better, have fewer psychosocial problems, and adhere more closely to treatment if they understand their diagnosis and the proposed therapy. Parental requests for secrecy are particularly difficult when adolescents are capable of making health care decisions. Generally, physicians can persuade parents to allow disclosure of information to the child, provide developmentally appropriate information, and help the child cope with the news. One study of children who died of cancer found that no parent regretted talking with their child about death, but many parents who did not do so later regretted it (9).

Physicians should give forthright answers when children ask directly about their diagnosis. Deceiving the child would compromise the physician's integrity and patients' trust in the medical system.

CONFIDENTIALITY

Exceptions to Confidentiality

Physicians and other health care workers must report cases of suspected child abuse or neglect to child protective services agencies. Confidentiality is overridden to protect vulnerable children from a high likelihood of serious harm. To be justified in reporting a case, physicians do not need definitive proof of abuse and neglect, but only sufficient information to warrant a fuller investigation. In evaluating possible cases of child abuse, physicians should treat parents with respect, keeping in mind that most parents are trying their best to deal with a difficult situation. Intervention might enable parents to obtain enough assistance and support to prevent further abuse. In extreme cases, protective service agencies may remove the child from parental custody.

Disclosure to Schools

Physicians might need to disclose health information to schools. Whenever information is disclosed, physicians should disclose only information that is truly needed. For example, a school does not need to know the diagnosis, but only that the child's absence was medically indicated. Doctors might also need to arrange for the child to receive medications at school. It is useful for physicians to discuss how parents, the child, and school personnel might respond to inquiries about the child's health in ways that maintain confidentiality.

Adolescents

Adolescents commonly wish to keep certain information confidential from their parents—for instance, that they are receiving care for mental health, STDs, pregnancy, or substance abuse (10, 11). Assurances

of confidentiality increase the willingness of adolescents to seek needed health care, particularly for such sensitive conditions, and to disclose information candidly to physicians. Concerns about confidentiality deter adolescents from seeking needed care. Hence, physicians should routinely discuss confidentiality with adolescents and offer them an opportunity to talk privately, apart from parents.

Many physicians provide absolute rather than conditional assurances of confidentiality (12). As Chapter 5 discussed, however, overriding confidentiality is ethically appropriate and legally mandated in several situations. Moreover, most adolescents believe that confidentiality should be overridden when a patient plans to commit suicide or has been physically or sexually abused. Thus, physicians should explain that confidentiality is not absolute and that exceptions are made in specific situations (10).

Even when adolescents are allowed to consent to treatment for sensitive conditions, parents' involvement in their subsequent care will generally be beneficial. State laws on informing parents of the adolescent's care vary both according to the condition and by state (13). For some sensitive conditions, physicians are required to notify parents or are permitted (but not required) to do so. In other conditions, physicians are prohibited from informing parents without the minor's consent. In still other conditions, doctors may use their judgment about disclosing to parents.

Generally, physicians should encourage adolescents to discuss medical decisions with their parents, who usually provide useful support and advice (11). It is often impossible to keep the parents from learning about the child's condition because of the condition's nature, the practicalities of obtaining treatment, or the need to pay for care. Doctors can offer to help adolescents disclose information to their parents. In some situations, disclosure might be counterproductive or dangerous, as when domestic violence is likely. In such situations, it would be desirable for the adolescent to confide in a trusted adult relative.

REFUSAL OF MEDICAL INTERVENTIONS

DISAGREEMENTS BETWEEN PARENTS AND PHYSICIANS

Parents sometimes refuse care that physicians believe is in the child's best interests or provide suboptimal care for the child at home. Doctors need to try to persuade parents to accept effective interventions that have few side effects (*see* Chapter 4). In addition, physicians, together with social workers and nurses, can mobilize emotional support and social resources to help the parents provide better care. In rare situations, physicians should ask the courts to override parental decisions—for example, when parents cannot be persuaded to accept life-saving therapy that has few side effects, such as antibiotics for bacterial meningitis in a previously healthy child. If disagreements persist, then the physician's response to parents' refusal of treatment will depend on the clinical circumstances, the benefits and burdens of treatment, and, in some cases, on the child's wishes.

INTERVENTIONS OF LIMITED EFFECTIVENESS OR GREAT BURDENS

Physicians should respect parents' informed refusals of interventions that have limited effectiveness, impose significant side effects, require chronic treatment, or are controversial. Such interventions are not clearly in the child's best interest.

EFFECTIVE INTERVENTIONS WITH FEW SIDE EFFECTS

Parents sometimes refuse treatments that are highly effective in restoring a child with life-threatening illness to health, are short term, and have few side effects (14). For example, Jehovah's Witnesses commonly refuse blood transfusions for children who undergo major trauma. Christian scientist parents often refuse antibiotics for life-threatening bacterial infections. Physicians who are unable to persuade parents to accept such interventions should seek a court order to administer the treatment (15). A court order is important because it signifies that society regards the parent's refusal as unacceptable. As one court declared, although "parents may be free to become martyrs themselves," they are not free to "make martyrs of their children" (16).

Overriding parents through the courts should be a last resort. Even if a child with asthma or diabetes is not receiving medications regularly, disrupting the parent–child bond causes emotional distress for the child. Foster placement or institutionalization might be worse for the child than care from well-meaning parents who are trying to cope with difficult circumstances.

For parental refusal of vaccinations, see Chapter 44.

EFFECTIVE BUT BURDENSOME THERAPY

Parents sometimes refuse interventions that cure a fatal disease in the vast majority of cases but highly burdensome, such as bone marrow transplantation in acute lymphocytic leukemia or combination chemotherapy in testicular carcinoma. If parents continue to refuse such therapy after repeated attempts at persuasion, some physicians seek court orders to compel treatment (17). In doing so, physicians need to take into account the need for long-term parental cooperation with the child's care. Physicians should listen to the parents' objections and show respect for their opinions and ongoing responsibility for the child. In situations where treatment is less successful, the physician's obligation to advocate for it is weaker.

ADOLESCENT'S REFUSAL OF INTERVENTIONS

In some cases, adolescents refuse effective treatments. The physician's response should depend on the seriousness of the clinical situation, the effectiveness and side effects of treatment, the reasons for refusal, the parents' preferences on treatment, and the burdens of insisting on treatment. It is difficult to force adolescents to take ongoing therapies, such as insulin shots for diabetes or inhalers for asthma. The most constructive approach is to try to understand the reasons for refusal, to address them, and to provide psychosocial support. In several cases, adolescents have run away from home rather than accept potentially curative cancer chemotherapy that has significant side effects (18). Because it is physically difficult, as well as morally troubling, to force such treatment on adolescents, these refusals have been accepted, particularly when the parents have supported the child's refusal.

INTERVENTIONS WITH NO MEDICAL INDICATIONS

Parents sometimes request medications to modify their child's behavior or enhance their school performance. Stimulant medications improve distractibility, inattention, and impulsivity in children who do not meet criteria for attention deficit/hyperactivity disorder (ADHD). If such use of stimulant medications is widespread, parents may feel pressured to use them so that their children are not at a disadvantage.

Critics contend that better alternative to such use of medications is instruction and practice to strengthen the will of a restless and unruly child (19). Proper moral education requires shaping of character. These critics contend that unlike other parental steps to help their children, such as tutoring, medications rupture the bond between effort and accomplishment and undermine the child's responsibility, self-control, and sense of right and wrong.

In rebuttal, other writers point out that these critics create a false dichotomy between medication and effort. In fact, students using stimulant medications, like drinking coffee, still must study hard to learn. It is also an oversimplification to suggest that poor school performance results primarily from a lack of will and effort. In a child with significant behavioral and learning problems who does not meet criteria for ADHD, if behavioral and counseling approaches prove ineffective, it is reasonable for informed parents to carry out a trial of medications.

HANDICAPPED INFANTS

Premature infants with low birth weight can be treated effectively in neonatal intensive care units (NICUs) and subsequently achieve normal growth and development. At extremely low birth weights, however, such as below 400 g, very few infants survive even with intensive care, and survivors commonly have severe neurologic disabilities.

The 1985 federal “Baby Doe Regulations” set limits on decisions to withhold medical treatment from disabled infants less than 1 year old. They are intended to ensure interventions such as surgery for tracheal–esophageal fistula in infants with Down syndrome. Under these regulations, treatment other than “appropriate nutrition, hydration, or medication” need not be provided if (a) the infant is irreversibly comatose, (b) treatment would merely prolong dying, (c) treatment would not be effective in ameliorating or correcting all life-threatening conditions, (d) treatment would be futile in terms of survival, or (e) treatment would be virtually futile and would be inhumane. Decisions to withhold medically indicated treatment may not be based on “subjective opinions” about the child’s future quality of life. Subsequently, the Born-Alive Infants Protection Act of 2002 was intended to reject the idea that a child’s care should vary according to “whether that child’s mother or others want him or her” (20).

The Baby Doe Regulations have been sharply criticized, for excluding parents from decision making (20–22). A further criticism is that maximal medical interventions must be provided unless they are futile or the child is irreversibly comatose or dying. In other clinical settings, the mere possibility of survival does not require the physicians and families to employ all available medical technology (23).

SUMMARY

1. Parents are generally given great discretion to make decisions for their children, on the assumption that they will act in the child’s best interests. Generally, parents and physicians should make shared decisions about the child’s care.
2. A parent may not forego interventions that are almost always life-saving and have few serious adverse effects.
3. Parents may refuse interventions that have a very low likelihood of success and great adverse effects.
4. As children gain maturity, they should play an increasing role in decision making, as appropriate for their developmental stage.

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